This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the administration of Aciclovir and Valaciclovir for the treatment of Genital Herpes Management

Summary of NICE / NICE CKS Guidelines for Genital Herpes Management

Overview

- **Genital herpes (HSV-1 and HSV-2):** A lifelong infection with unpredictable recurrences. Most people have mild to moderate symptoms, but complications can occur.
- **Transmission:** Sexual contact, highest risk during prodrome and active lesions. Can be transmitted even without symptoms (asymptomatic shedding).

Assessment

- **Clinical presentation:** Painful vesicles progressing to shallow ulcers, fever, malaise, dysuria, inguinal lymphadenopathy.
- **Viral swab:** PCR or culture for confirmation (vesicular fluid or base of ulcer).
- **Sexual history and risk factors:** Number of partners, contraception use, STI screening, frequency of recurrences (determine if suppressive therapy indicated).
- **Frequency of recurrences:** 6 or more per year = consider suppressive therapy.

Management

- **First episode:** Antiviral therapy reduces healing time and symptom severity. Continue for 5-10 days.
- **Recurrent episodes:** Most effective if started within 48 hours of symptom onset (vesicle or prodrome). Shorter course than first episode.
- **Suppressive therapy:** If 6 or more recurrences per year, consider long-term daily antivirals (aciclovir 400mg BD or valaciclovir 500mg daily).
- **Supportive care:** Analgesia, topical treatments (Vaseline), keep lesions clean and dry.

Red Flags / Complications

- **Immunocompromised patients:** Higher risk of severe/disseminated disease. May need IV aciclovir. Refer to specialist.
- **Pregnancy near term (>34 weeks):** Risk of neonatal herpes. Maternal antivirals reduce transmission. Caesarean section advised if lesions present at delivery.
- **Meningitis/encephalitis:** Rare. Headache, neck stiffness, altered consciousness. Emergency referral required.
- **Disseminated infection:** Rare. Systemic symptoms, hepatitis, multi-organ involvement. Specialist care.

Patient Group Direction

for the supply/administration of

Aciclovir 400mg tablets

for the treatment of

Genital Herpes Management

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Treatment of first episode and recurrent genital herpes simplex infection. Suppressive therapy for frequent recurrences (6 or more per year).
Inclusion criteria	Confirmed or highly suspected genital herpes (HSV-1 or HSV-2 positive or clinical diagnosis) Aged 16 years and over Able to give informed consent No contraindications or cautionable conditions (see exclusions/cautions)
Exclusion criteria	Hypersensitivity to aciclovir or valaciclovir Significant renal impairment (eGFR <30 mL/min; caution if eGFR 30-60) Severe hepatic impairment Immunocompromised patients with disseminated infection or

	suspected meningitis/encephalitis (refer to specialist for IV therapy) Neonatal herpes risk (pregnant women near term; refer to specialist)
Cautions	Renal impairment (eGFR 30-60 mL/min): dose adjustment may be required Elderly patients: increased risk of renal impairment; adequate hydration essential Hepatic impairment (mild-moderate): monitor closely Dehydration: ensure adequate fluid intake; risk of crystalline nephropathy if dehydrated Neurological disease: rare but possible (confusion, tremor, hallucinations) particularly with IV formulation or renal impairment
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Aciclovir 400mg tablets
Legal category	POM
Storage	Below 25°C. Protect from light and moisture. Keep in original container.
Route/method of administration	Oral, swallowed whole with water
Dose and frequency	First episode: 400mg three times daily for 5-10 days (depending on severity and response) Recurrent episode: 800mg three times daily for 2 days (starting within 48 hours of symptom onset) Suppressive therapy (6+ recurrences/year): 400mg twice daily; continue for 6-12 months then review
Quantity to be administered and/or supplied	Up to 15 tablets per course (first episode, 5 days at 400mg TDS) or up to 30 tablets (10-day course) for severe first episodes
Maximum or minimum treatment period	First episode or recurrence: 2-10 days. Suppressive therapy: ongoing (typically 6-12 months).
Adverse effects	Common (>1%): nausea, headache, diarrhoea, abdominal pain, rash (including photosensitivity) Uncommon (0.1-1%): vomiting, fatigue, fever, arthralgia Rare (<0.1%): renal impairment (elevated creatinine, crystalline nephropathy if severe dehydration), neurological effects (confusion, tremor, hallucinations, psychosis) usually with IV therapy or in renal

	failure, thrombocytopenia, hepatitis Yellow card report suspected adverse reactions via https://yellowcard.mhra.gov.uk

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with aciclovir or valaciclovir tablets.
Follow-up advice to be given to patient or carer	This is not a cure for herpes; the virus remains in the body and recurrences are possible. Use condoms consistently to reduce transmission to partners, even if no symptoms present. Avoid all sexual contact during prodromal symptoms and active lesions (vesicles or ulcers). Discuss disclosure to sexual partners; consider partner testing and treatment. Consider HPV vaccination if not already vaccinated (if cervical screening due, attend regularly). If pregnant or planning pregnancy, discuss genital herpes management with GP/obstetrician early. Complete the full treatment course even if symptoms improve early.

	Seek medical advice if symptoms do not improve within 10 days, or if you experience severe pain, fever, or signs of systemic infection. Report any adverse effects via Yellow Card (https://yellowcard.mhra.gov.uk). If recurrences become frequent (6+ per year) discuss suppressive therapy with healthcare provider.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- NICE CKS Guidance: Herpes simplex virus infections
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		

1/11/25	31/10/26		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	**Qualification**	**Signature**	**Date**
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	**Change details**	**Date**
001	Development & issue of new PGD	1st November 2025

Patient Group Direction

for the supply/administration of

Valaciclovir 500mg tablets

for the treatment of

Genital Herpes Management

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Alternative antiviral for first episode and recurrent genital herpes simplex. Better bioavailability than aciclovir (less frequent dosing). Suppressive therapy for frequent recurrences (6 or more per year).
Inclusion criteria	Confirmed or highly suspected genital herpes (HSV-1 or HSV-2 positive or clinical diagnosis) Aged 16 years and over Able to give informed consent No contraindications or cautionable conditions (see exclusions/cautions) Preferred in patients requiring less frequent dosing (twice daily vs three times daily)

Exclusion criteria	Hypersensitivity to valaciclovir or aciclovir Significant renal impairment (eGFR <30 mL/min; caution if eGFR 30-60) Severe hepatic impairment Immunocompromised patients requiring high-dose therapy; refer to specialist Neonatal herpes risk (pregnant women near term; refer to specialist)
Cautions	Renal impairment (eGFR 30-60 mL/min): dose adjustment required; seek specialist advice if eGFR <30 Elderly patients: increased risk of renal impairment; ensure adequate hydration Hepatic impairment (mild-moderate): monitor closely Thrombotic thrombocytopenic purpura (TTP) / haemolytic uremic syndrome (HUS): rare risk in immunocompromised patients Adequate hydration essential to reduce renal toxicity risk
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Valaciclovir 500mg tablets
Legal category	POM
Storage	Below 30°C. Protect from light and moisture. Keep in original container.
Route/method of administration	Oral, swallowed whole with water
Dose and frequency	First episode: 500mg twice daily for 5-10 days Recurrent episode: 500mg twice daily for 3-5 days (starting within 48 hours of symptom onset) Suppressive therapy (6+ recurrences/year): 500mg once daily; continue for 6-12 months then review
Quantity to be administered and/or supplied	Up to 20 tablets per course (first episode or recurrence, up to 10 days at 500mg BD)
Maximum or minimum treatment period	First episode or recurrence: 3-10 days. Suppressive therapy: ongoing (typically 6-12 months).
Adverse effects	Common (>1%): headache, nausea, dizziness, abdominal pain, rash Uncommon (0.1-1%): vomiting, fatigue, arthralgia, tremor

	Rare (<0.1%): renal impairment (elevated creatinine, crystalline nephropathy), neurological effects (confusion, hallucinations, psychosis), thrombotic thrombocytopenic purpura (especially immunocompromised), hepatitis, thrombocytopenia Yellow card report suspected adverse reactions via https://yellowcard.mhra.gov.uk

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with aciclovir or valaciclovir tablets.
Follow-up advice to be given to patient or carer	This is not a cure for herpes; the virus remains in the body and recurrences are possible. Use condoms consistently to reduce transmission to partners, even if no symptoms present. Avoid all sexual contact during prodromal symptoms and active lesions (vesicles or ulcers). Discuss disclosure to sexual partners; consider partner testing and treatment. Consider HPV vaccination if not already vaccinated (if cervical screening due, attend regularly). If pregnant or planning pregnancy, discuss genital herpes

	management with GP/obstetrician early. Complete the full treatment course even if symptoms improve early. Seek medical advice if symptoms do not improve within 10 days, or if you experience severe pain, fever, or signs of systemic infection. Report any adverse effects via Yellow Card (https://yellowcard.mhra.gov.uk). If recurrences become frequent (6+ per year) discuss suppressive therapy with healthcare provider.

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